SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

# Regorafenib (Stivarga) monotherapy

Previously treated metastatic colorectal cancer · NCCP 00244 · version 5 · 28-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
Regorafenib is a targeted tablet treatment for colorectal cancer that has spread, after previous treatments or when those treatments are unsuitable. The aim is to control the cancer. Your team discusses the likely benefit, side effects, alternatives and your priorities.

**Schedule**
Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Regorafenib**
ORAL MULTIKINASE INHIBITOR

- ☐ Hand-foot skin reaction: painful, red, swollen or blistered hands and feet
- ☐ Fatigue, reduced appetite, weight loss and mouth soreness
- ☐ Diarrhoea, nausea, vomiting and dehydration
- ☐ Raised blood pressure and changes in thyroid function
- ☐ Liver injury, sometimes severe
- ☐ Infection, anaemia, low platelets and bleeding
- ☐ Uncommon but serious heart, bowel, neurological or blood-vessel problems
- ☐ Impaired wound healing and harm to an unborn baby

Discuss the colorectal disease-control aim, likely benefit and uncertainty, alternatives including supportive care without regorafenib, early hand-foot skin reaction, blood-pressure and liver monitoring, fatigue, diarrhoea and appetite loss, serious bleeding/infection/heart/bowel risks, wound healing, interactions, pregnancy prevention and the option to reconsider. Agree who authorises changes and restarts.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00244 v5.

SACTCheck v0.84.34
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